February 20, 2026

Steven Posnack
Principal Deputy Assistant Secretary for Technology Policy
Assistant Secretary for Technology Policy (ASTP) and the Office of the National Coordinator for Health Information Technology (ONC)
Attention: HHS Health Sector AI RFI
Mary E. Switzer Building
Washington, DC 20201

RE: *Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care*

Submitted electronically

Dear Mr. Posnack,

Tempus AI, Inc. ("Tempus") appreciates the opportunity to submit comments in response to ASTP/ONC's request for information on accelerating the adoption and use of artificial intelligence as part of clinical care.

Tempus is a technology company at the forefront of advancements in precision medicine. Tempus offers a wide range of next-generation sequencing (NGS) tests to support pharmaceutical and biotechnology development, clinical studies, and patient treatment, including DNA and RNA tissue-based and liquid biopsy assays. Tempus believes that healthcare will be transformed by the application of technology. We are increasingly concerned that structural factors in the healthcare system will slow that technological progress. We believe there is an opportunity to address the concerns that could limit technological advancement and instead promote the safe and effective use of AI to benefit patients.

Regulation - Current Impact and Recommendations

Risk-Based Approach

A clear, predictable, and risk-based regulatory framework is paramount. We strongly advocate for a risk-based approach to regulation, where the level of regulatory oversight is proportional to the risk of the AI tool, inclusive of balancing pre- and post-market requirements and accepting trade-offs of sensitivity/specificity for low-risk medical devices that bring to market novel medical insights while maintaining a physician in the loop. This will enable more agile regulation of lower-risk tools while ensuring that higher-risk tools undergo appropriate pre-market review and post-market surveillance.

Reimbursement - Current Impact and Recommendations

Government's Role in Coding for Medical Technology

A barrier in the current system is the process for obtaining new Current Procedural Terminology (CPT) codes, which is governed by the American Medical Association (AMA). The AMA has long played a crucial role in standardizing the language of medicine for billing and reimbursement, and its CPT code set is the bedrock for how physicians are paid for their services and for how medical devices are reimbursed. The AMA has clarified that the only existing code pathway for AI technologies is a Category III code, which by definition are emerging technologies. Additionally, Category III codes have no defined payment mechanism. While the AMA has expressed interest in creating a new code set applicable to AI technologies, there is no alignment of this code set with a Medicare payment pathway or fee schedule. This creates a circumstance where technologies may exist but have no corresponding code, and the only coding pathway is one that has no associated payment. This lack of a clear payment pathway disincentivizes physicians and health systems from adopting the technology, as it represents uncompensated work and investment. In turn, technology developers, seeing a limited and uncertain path to reimbursement despite going through all the appropriate channels, are discouraged from investing the massive resources required to bring these tools to life. To break this cycle of misaligned interests, the federal government should step in to create a modernized, impartial pathway for reimbursement that is specific to AI technology.

Aligning Interests – Technology Companies, Providers, and Government

To encourage companies to invest in bringing AI to healthcare, and to promote mass adoption, we need a system that aligns interests and provides incentives to reduce overall costs in our healthcare system. The system should provide the upfront financial incentives necessary for tech developers to innovate and for physicians to adopt new tools. It should encourage a data-driven, value-based approach to ensure that the most effective and efficient technologies become a permanent part of the healthcare landscape, improving patient care and saving long term costs. By having the government step in to create this provisional bridge, we can accelerate the adoption of high-value AI, foster competition, and ultimately modernize our healthcare system to be more effective and affordable for everyone. The system should tie reimbursement to proven value. For certain technologies, developers should first secure FDA authorization and then receive provisional coverage and reimbursement, encouraging investment and adoption. During this provisional period, developers can negotiate reimbursement rates and collect real-world evidence to demonstrate improved patient care and cost savings in order to maintain long-term reimbursement.

Responses to Specific Questions

1. What are the biggest barriers to private sector innovation in AI for health care?

The most significant barriers include:

- Ambiguity in the regulatory pathway for certain AI-based products can deter investment and innovation in the US market. Specific concerns include the status of non-device clinical decision support software, positions on enforcement discretion which could change, and potential regulation of more advanced non-determinate medical device software such as Foundation Models or generative AI where stochastic outputs present challenges for traditional verification and validation frameworks.

- Access to large, high-quality, and diverse datasets is essential for training/validating AI models, and for effective post-market monitoring/improvement. However, data is often siloed in disparate electronic health record (EHR) systems, and a lack of interoperability hinders data sharing.
- Regulatory requirements for AI-based medical devices should be commensurate with their risks and context of use. For example, The IMDRF document titled Software as a Medical Device: Possible Framework for Risk Categorization and Corresponding Considerations is specifically referenced in the FDA Guidance document titled Clinical Decision Support Software Guidance for Industry and Food and Drug Administration Staff Document issued on January 29, 2026. This risk categorization framework describes how the significance of information provided by medical device software to a healthcare decision (i.e., to treat/diagnose, to drive clinical management, or to inform clinical management) results in a different benefit-risk profile, and this should be taken into consideration when defining pre-market submission requirements.
- FDA should apply a reasonable standard for software performance for lower-risk Class II clinical decision support medical devices. Balancing pre- and post-market requirements and acknowledging necessary trade-offs of sensitivity vs. specificity for low-risk medical devices is essential to unlock significant value to hospitals and patients alike, provided a physician remains the ultimate decision maker.

2. What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care and why? What HHS regulations, policies, or programs could be revisited to augment your ability to develop or use AI in clinical care? Please provide specific changes and applicable Code of Federal Regulations citations.

As described in Docket No. FDA‐2025‐P‐5560 (Medical Devices; Exemption From Premarket Notification: Radiology Computer-Aided Detection and/or Diagnosis Devices and Computer-Aided Triage and Notification Devices), Tempus supports the proposal for partial 510(k) exemption for the listed medical devices. We further recommend the expansion of this partial exemption to include:

- Product Code QIH/LLZ: Radiological Machine Learning Based Quantitative Imaging Software with Change Control Plan (potentially through updates to the special controls for 21 CFR § 892.2055).
- Product Code QPN: Software Algorithm Devices to Assist Users in Digital Pathology (through updates to special controls for 21 CFR § 864.3750).
- Product Code PSY and QKQ: Whole Slide Imaging System, and Digital Pathology Image Viewing and Management Software (through updates to special controls for 21 CFR § 864.3700)

3. How can HHS best support private sector activities (e.g., accreditation, certification, industry-driven testing, and credentialing) to promote innovative and effective AI use in clinical care?

For medical devices regulated by FDA, the use of new accreditation/certification/credentialing programs created by HHS or third parties imposes redundant burdens on clinical adoption with limited additional value given the robustness of the current FDA process. HHS should not implement or encourage such additional steps for medical devices. To support a clear, predictable, and risk-based regulatory framework, FDA should continue engagement with standards development organizations and focus on the recognition of appropriate standards alongside guidance document development activities for pre- and post-market requirements for AI-based medical devices. Harmonization of requirements across international regulatory bodies will further support industry investment in innovative and effective AI when requirements for market access are standardized and streamlined.

4. Where have AI tools deployed in clinical care met or exceeded performance and cost expectations and where have they fallen short? What kinds of novel AI tools would have the greatest potential to improve health care outcomes, give new insights on quality, and help reduce costs?

At the end of 2025, total healthcare costs in this country were estimated to exceed $5.7 trillion, which consumes almost 20% of the entire US economy, with a growth rate of approximately 7%. By most estimates, avoidable waste accounts for 20-30% of total healthcare spend, with some estimates as high as 40%. At the lower end, if we could mitigate errors, mistakes, and other waste, we would save approximately $1.5 trillion annually. To-date, AI tools have fallen short of addressing this expense. A change to regulatory and reimbursement pathways to incentive investment into AI solutions in this space could unlock significant cost savings.

Additionally, there is a vast amount of unused data generated as part of routine care, which can be considered the “digital exhaust” of the healthcare system, because this data is generated as a byproduct of routine digital health interactions and activities. This includes a wide range of information such as EHRs, digital imaging, lab results, and other forms of digital documentation that are created during routine clinical care. There is an opportunity to greatly improve healthcare outcomes by leveraging AI to extract information from this digital exhaust, identifying patterns that are not readily identifiable by clinicians, to provide otherwise unavailable insights into a patient’s health status. Again, an alignment of regulatory and reimbursement pathways is critical to unlocking investment in these solutions.

We appreciate the opportunity to provide these comments, and we thank you for your consideration.

Sincerely,

Lauren Silvis
Senior Vice President External Affairs
Tempus AI